Centers for Medicare & Medicaid Services (CMS)
Assistant Secretary for Technology Policy (ASTP) / Office of National Coordinator for Health Information Technology (ONC)

Health Technology Ecosystem

Response to Request for Information (RFI) for 2025-08701 (90 FR 21034) | File Code: CMS-0042-NC

Submission Date: June 9, 2025

Submitted By:
iVision, Inc. (DBA iVision Consulting, Inc.)
12300 Twinbrook Pkwy, Suite 206
Rockville, MD 20852
UEI: M33LHLK4UN85
CAGE Code: 5M6T2

Submitted To:
CMS, ASTP / ONC

Point of Contact:
Mr. Pallav Thakkar, President and CEO
Email: Pallav.Thakkar@ivisionconsulting.com | Phone: (301) 529-8775
www.ivisionconsulting.com

This proposal includes data that shall not be disclosed outside the Government and shall not be duplicated, used, or disclosed—in whole or in part—for any purpose other than to evaluate this proposal. If, however, a contract is awarded to this offeror as a result of—or in connection with—the submission of this data, the Government shall have the right to duplicate, use, or disclose the data to the extent provided in the resulting contract. This restriction does not limit the Government's right to use information contained in this data if it is obtained from another source without restriction. The data subject to this restriction are contained in all pages of the proposal.

TABLE OF CONTENTS

1 GENERAL VENDOR INFORMATION 1
2 RESPONSE TO QUESTIONS FOR: B. PATIENTS AND CAREGIVERS 1
2.1 Response to PC-1 1
2.2 Response to PC-2 2
2.3 Response to PC-3 3
2.4 Response to PC-4 3
2.5 Response to PC-5 4
2.6 Response to PC-6 5
2.7 Response to PC-7 5
3 RESPONSE TO QUESTIONS FOR: E. TECHNOLOGY VENDORS, DATA PROVIDERS, AND NETWORKS 5
3.1 Response to TD-1 5
3.2 Response to TD-4 6
3.3 Response to TD-12 7

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.

1 General Vendor Information

iVision, Inc. (DBA iVision Consulting, Inc.) is an award-winning HUBZone and Small Disadvantaged Business (SDB) that delivers innovative technology solutions and services with a strong focus on Health IT. Since 2007, iVision has supported the Department of Health and Human Services (HHS), including the National Institutes of Health (NIH), as well as commercial health organizations, providing mission-aligned solutions that drive digital transformation across the healthcare ecosystem. Our capabilities include artificial intelligence (AI), machine learning (ML), advanced data analytics, cloud services, enterprise architecture, software development, and business process optimization, all tailored to meet the evolving needs of the health sector.

iVision Consulting is excited to provide comments in response to the Request for Information (RFI) from the Center for Medicare and Medicaid Services (CMS) and the Assistant Secretary for Technology Policy (ASTP) Office of National Coordinator for Health Information Technology (ONC). We strongly support the agencies' commitment to building on the existing policy framework to advance the large-scale adoption of health management and care navigation applications. We share the vision of reducing barriers to data access and exchange, realizing the potential of emerging innovations, and accelerating the transition to a patient-centered, learning health system. As an experienced Health IT firm with deep experience supporting HHS and commercial healthcare clients, we appreciate the opportunity to contribute to this important dialogue and offer our perspective on what is working well in today's digital health ecosystem, where there are opportunities for improvement, and how future policy and rulemaking can promote innovation, engagement, and better health outcomes.

2 Response to Questions for: B. Patients and Caregivers

2.1 Response to PC-1

PC.1 What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

Patients need integrated apps that help them manage chronic conditions, navigate care, access their medical history across providers, and obtain real-time guidance. The most impactful health management and care navigation apps for Medicare beneficiaries would be AI-powered, personalized platforms that evolve with a patient's needs, particularly for chronic condition management, medication adherence, mental health check-ins, and wellness goals like weight loss or activity tracking.

Top things digital health products should enable:

- Create a personalized health plan that adjusts in real time based on input (steps, symptoms, glucose levels, food tracking, mental health, etc.).
- Offer medication reminders with adherence feedback, side effect education, and refill management, and contraindications with other medications or foods.
- Automate care coordination tasks like finding specialists, scheduling appointments, sending reminders, and managing pre-authorizations.
- Track and visualize progress toward health goals (e.g., blood pressure, A1C, weight, mental well-being).
- Integrate and interpret claims data to flag unnecessary duplicate testing, recommend lower-cost providers, or identify eligibility for care programs.
- Provide AI-powered health coaches that help users build habits and motivate them through daily nudges, positive reinforcement, and simple, human-like check-ins which can be customizable to the beneficiaries' preferences.

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.

These are features that a beneficiary may want or need from a personal digital assistant:

- Track health goals (e.g., "Remind me to walk 6,000 steps a day and adjust if I hit a plateau.").
- Notice changes in patterns (e.g., "You have not logged your blood pressure in 3 days. Is everything okay?").
- Coach through hard moments, like when craving unhealthy food or skipping exercise.
- Communicate directly with my care team, alerting them if they miss multiple doses or my readings change significantly.
- Simplify insurance navigation, including coverage clarifications, prior authorizations, or appeals.
- Speak plainly, explaining labs, procedures, and billing codes in a human, easy to understand, voice.

2.2 Response to PC-2

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)? If so, what are some examples of benefits it has provided? If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information? Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

No, many do not have easy access to their own or loved ones' health information in one location. Each provider usually has their own portal with different login systems, layouts, and levels of information. Insurance plans are yet another system, completely disconnected from the patient's actual care experience, requiring a separate account, separate app, and a mental spreadsheet to track it all.

A unified single system portal that pulls in data from providers, payers, labs, and pharmacies would be a game-changer, especially when managing chronic conditions, preparing for specialist visits, or caring for aging family members. Imagine being able to:

- See past labs, imaging, diagnoses, and medications in one timeline.
- Upload or receive second opinions from providers without faxing or scanning.
- Coordinate care between specialists and primary care with shared notes and referrals.
- Quickly share a comprehensive, up-to-date record during emergencies.
- Track the financial impact of care (deductibles, copays, authorizations) alongside clinical details.

What is missing and why it matters - Key data types often unavailable or siloed include:

- Imaging results (x-rays, MRIs, CT scans).
- Provider notes (including discharge summaries and mental health observations).
- Specialist reports, particularly from out-of-network or one-time visits.
- Billing history and real-time cost estimates.

The barriers include:

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.

- Lack of interoperability between EHRs.
- Limited or delayed sharing of data between payers and providers.
- Poor UX/UI that frustrates older adults or those with limited digital literacy.
- Separate logins for every provider, hospital system, and insurance portal.

It is overwhelming and deeply inefficient, not just for the patient, but for the system as a whole. CMS could lead the way by incentivizing the creation of a Medicare-backed Health Hub that aggregates, simplifies, and humanizes health data. It should allow beneficiaries to connect third-party apps, delegate access to caregivers, and get contextual explanations about their care, all from one place.

2.3 Response to PC-3

PC-3. Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?

There are several health management and personal health record apps that could be incredibly useful to Medicare beneficiaries and their caregivers, but many of them are either underutilized, too complex, or not Medicare-friendly out of the box. Some promising examples:

- Apple Health / Google Health Connect: These apps allow users to consolidate data from multiple providers, fitness trackers, and health systems, but integration with Medicare data and claims is limited or non-existent.
- MyChart: While widely used, it is fragmented by health system, users often end up with multiple logins and disconnected portals, which defeats the purpose of centralization.
- Caregiver-specific apps like CaringBridge or CareZone offer organizational tools for medication tracking and appointment notes but are not well-integrated with actual medical records.
- AI-powered tools like Lark, Noom, or Wellory show strong potential for personalized coaching around behavior change, weight loss, and chronic care, but most are tailored to commercial plans, not Medicare.

CMS could consider setting up a trusted, centralized, CMS-endorsed platform where these apps can securely plug in, share data, and offer coordinated insights. Beneficiaries should not have to switch multiple apps to manage their health. With better integration, oversight, and user education, these kinds of tools could dramatically improve quality of life, reduce caregiver stress, and prevent unnecessary hospitalizations.

2.4 Response to PC-4

PC-4. What features are missing from apps you use or that you are aware of today? a. What apps should exist but do not yet? Why do you believe they do not exist yet? b. What set of workflows do you believe CMS is uniquely positioned to offer?

Today's health apps are fragmented, reactive, siloed, and often not designed with older adults or caregivers in mind. Even the most advanced tools are missing the human-centered features that would truly make healthcare navigable, personalized, and empowering. What features are missing:

- True multi-provider integration: Apps still operate in silos. A patient seeing multiple specialists across different systems has no unified view.

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.

- AI-driven coaching based on CMS claims or medical data. Imagine an app that says, "Based on your recent labs and visit history, let's work on blood sugar today."
- Contextual education: When you get a new diagnosis or medication, apps should explain what that means in plain language, tailored to your condition and age group.
- Caregiver delegation features: It is often impossible or legally unclear how to give a loved one access to your health app, even with consent.

What apps should exist but do not yet:

- A Medicare Health Companion app, which could act as a GPS for healthcare. It would guide users through benefits, appointments, preventative screenings, billing questions, and more—all in one place, with real-time updates.
- A digital medication coach for polypharmacy patients, particularly those on 5+ prescriptions, integrated with pharmacy and provider systems to flag risks, missed doses, or dangerous interactions.
- A Caregiver Dashboard that lets authorized loved ones view a beneficiary's care journey, coordinate with other caregivers, and get alerts when something changes.

These apps do not exist yet because of regulatory complexity, lack of CMS-facilitated API access, and unclear incentives for commercial developers to build specifically for Medicare users, for whom are often perceived as a "difficult" demographic to design.

CMS is uniquely positioned to offer digital workflows that no single health system or private app can replicate, due to its comprehensive access to longitudinal patient data across settings, providers, and time. With this unparalleled visibility, CMS could play a catalytic role in shaping the future of personal health management by developing or licensing a Medicare-wide personal health record that aggregates claims, encounters, coverage details, and preventive care schedules. CMS could further enhance engagement by enabling automatic, personalized alerts when a beneficiary is due for routine screenings, vaccinations, or follow-up care. Seamless, one-click care navigation tools would allow beneficiaries to find in-network providers, view appointment availability, compare procedure costs, and initiate referrals within a unified interface. Finally, CMS could introduce an AI-driven benefits coach that translates complex Medicare rules into plain-language prompts, helping users understand and act on their coverage, such as locating a provider for a newly available durable medical equipment benefit. The infrastructure already exists; what is needed now is thoughtful integration, stitched together into a secure, accessible platform that empowers patients and caregivers to manage their health with confidence and clarity.

2.5 Response to PC-5

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

b. What changes would enable timely access to high quality CMS and provider generated data on patients?

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.

Medicare beneficiaries who are becoming increasingly tech savvy are eager to use digital tools to manage their health but often find the experience fragmented and confusing. CMS should play a proactive and patient-centered role in promoting adoption by launching wide-reaching education campaigns through trusted touchpoints like AARP, community pharmacies, and Medicare Advantage newsletters, while also offering small, meaningful incentives such as "digital health credits" or app rebates that encourage seniors and caregivers to try vetted tools. CMS should further establish a publicly accessible Medicare App Marketplace showcasing CMS-reviewed products that meet basic criteria for health outcome impact, user-friendly design (especially for those with low digital literacy), and interoperability with systems like Blue Button 2.0 and Data at the Point of Care. CMS should modernize data infrastructure by enabling real-time API access to provider and claims data, making it easier to share with third-party apps and caregivers. Reimbursement policies should also reward providers who use digital tools to actively engage patients. In today's healthcare landscape, access to digital tools is essential to accessing care itself and far too many Medicare beneficiaries remain locked out of the very systems designed to support their health and well-being.

2.6 Response to PC-6

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

To ensure digital health products are accessible and user-friendly for Medicare beneficiaries and caregivers, especially those unfamiliar with technology, CMS should prioritize simple, intuitive design. Features like large-font text, voice navigation, plain-language instructions, and visual cues should replace medical jargon. These tools must also work across smartphones, tablets, and even landline-connected devices for users without high-speed internet. Accessibility should include multilingual support, screen reader compatibility, and offline functionality for rural areas. CMS could further improve usability by launching a national "digital health orientation" program through libraries, senior centers, and Medicare counselors to help users set up and trust these tools. Caregivers also need secure, integrated permissions to manage their loved ones' health data. Ultimately, ease of use must be rooted in dignity, simplicity, and the everyday realities of aging, caregiving, and chronic illness.

2.7 Response to PC-7

PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

An effective way to collect real-world data is through opt-in, passive data collection built into the apps themselves. APIs can transmit anonymized outcome data, such as blood pressure readings, medication adherence, or fewer emergency room visits, directly to CMS dashboards. This should be combined with periodic, user-friendly surveys for patients and caregivers to gather feedback on usability, engagement, and perceived value. CMS should also incorporate provider feedback and claims data to assess outcomes and costs from clinical, experiential, and financial perspectives. Patients must retain control and visibility over how their data is used. Clear explanations and a simple opt-out process are essential to protect privacy and build trust. To encourage participation, CMS could offer small Medicare account credits, wellness points, or other meaningful rewards.

3 Response to Questions for: E. Technology Vendors, Data Providers, and Networks

3.1 Response to TD-1

TD.1 What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

To stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers, CMS should consider the following short-term and longer-term steps:

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.

Short-term (next 2 years):

- Clarify and streamline regulatory guidance pertaining to data access, API requirements, HIPAA, and compliance to support faster development and deployment of digital health tools.
- Expand access to standardized, FHIR-based APIs across Medicare programs to enable easier integration with third-party applications.
- Launch pilot programs or innovation challenges that provide funding, visibility, and technical support to developers focused on Medicare use cases.
- Promote early engagement opportunities with developers and startups through roundtables, sandbox environments, and open forums.
- Publicize use cases and success stories that demonstrate the value of digital health tools for Medicare beneficiaries and caregivers.

Longer-term (beyond 2 years):

- Adjust reimbursement models and value-based care programs to encourage the use of digital health tools that improve care and outcomes for Medicare beneficiaries.
- Strengthen digital skills and access to technology among beneficiaries and caregivers to help them confidently use health apps and tools.
- Provide clear, consistent policies that make it easier for data to move securely between systems and give individuals greater access to and control over their health information.
- Create a trusted process to recognize and highlight digital health products that are effective, secure, and well-suited for the Medicare population.
- Build stronger partnerships between CMS, developers, researchers, patient advocates, and other stakeholders to ensure digital health solutions are practical, patient-centered, and widely adopted.

3.2 Response to TD-4

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS should consider the following actions to improve and promote use of open, standards-based, publicly available APIs over proprietary ones:

- Incentivize use of open, standards-based APIs like HL7 FHIR by linking their use to program eligibility, value-based care initiatives, or grant funding. This creates a strong incentive for developers and health organizations to move away from proprietary solutions in favor of interoperable, open technologies.
- CMS should continue to adopt and publish its own APIs using open, well-established standards, ensuring they are well-documented, consistently updated, and accessible to the public.
- Require vendors and health IT systems to disclose when proprietary APIs are used and provide an explanation for why they are used. This will help identify gaps in standardization that still need to be addressed. Provide clear technical and reference guidance, as well as implementation support to ease the adoption process and reduce risk.
- Partner with standards organizations and the community. CMS should continue to collaborate with groups like HL7, the ONC, and other industry consortia to advance the development and adoption of FHIR-based implementation guides.

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.

- Support and enforce interoperability policies. In line with the 21st Century Cures Act and ONC's Information Blocking Rule, CMS can strengthen enforcement mechanisms and reward organizations that actively promote open data exchange. This helps level the playing field and encourages more vendors to embrace open APIs as the industry standard.

3.3 Response to TD-12

TD-12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Yes, CMS should consider endorsing non-CMS data sources and networks, such as health information exchanges (HIEs), community care hubs, and other trusted data platforms, but only if these platforms meet clear standards for data quality, interoperability, privacy, and utility. High-performing, non-CMS data sources can expand access to timely, relevant health information, reduce duplication of effort, and improve care coordination for Medicare beneficiaries. CMS should consider the following criteria or metrics for endorsement:

- Interoperability Compliance: The data source should adhere to nationally recognized interoperability standards (e.g., HL7 FHIR, USCDI) and be able to exchange data effectively with CMS systems and other networks.
- Data Quality and Completeness: The network should demonstrate consistent, high-quality data with minimal gaps, including timely updates, standardized terminologies, and accurate patient matching.
- Privacy and Security Protections: The organization must comply with HIPAA and other federal data protection regulations, with robust safeguards to ensure data privacy and security.
- Consumer Access and Transparency: The platform should enable patients and caregivers to access their data easily and support mechanisms for consent, data sharing, and correction of inaccuracies.
- Governance and Accountability: CMS should evaluate whether the organization has transparent governance structures, clear data use policies, and processes for resolving disputes or data integrity issues.
- Proven Impact and Use Cases: Preference could be given to data sources that have demonstrated real-world value, such as improving care coordination, reducing readmissions, supporting public health reporting, or enabling analytics for underserved populations.
- Accurate Representation: CMS should consider whether the data source includes diverse populations and addresses disparities in access, care, and outcomes, particularly in rural, underserved, or underrepresented communities.

Use or disclosure of data contained on this page is subject to the restriction on the title page of this proposal.